

# Strengthening Coping-Focused Support for People Living with HIV in Public Hospital Settings in India

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## PURPOSE

This brief asks how India's HIV counselling and peer support could be strengthened using evidence on coping and resilience. It draws on a study of 121 adults living with HIV at three public hospitals in north India (Nandra & Khurana, 2025) and on a review of NACO's ART, counsellor and Care and Support Centre guidelines. It recommends one controlled pilot. It does not assess how services are delivered in practice.

### THE GAP

- An estimated 24.67 lakh people were living with HIV in India in 2022 (NACO & ICMR-National Institute of Medical Statistics, 2023).
- Depression and anxiety are common in this population (Chaudhury et al., 2016).
- NACO already requires depression screening at ART centres and offers peer counselling through Care and Support Centres (CSCs).
- The suggested support group topics do not name coping skills or self-blame.

### THE EVIDENCE

- 121 adults living with HIV, recruited at three public hospitals in north India.
- Emotional support, positive reframing and information use went with higher resilience.
- Humour, venting and self-blame went with lower resilience.
- The associations are small to moderate and do not show cause and effect.

### THE PROPOSAL

- Approve a six-month controlled pilot of coping-focused support at three or four ART centres, and decide on wider rollout from pre-specified criteria.

## WHAT THE STUDY FOUND

The study surveyed 121 adults aged 18 to 45 living with HIV, recruited by convenience sampling at three public hospitals in north India (113 men, 8 women). Coping was measured with the Brief COPE (Carver, 1997) and resilience with the Brief Resilience Scale (Smith et al., 2008). Six coping strategies were significantly related to resilience (Figure 1). Emotional support ( $r = .32$ ), positive reframing ( $r = .23$ ) and information use ( $r = .18$ ) were associated with higher resilience. Humour ( $r = -.23$ ), venting ( $r = -.20$ ) and self-blame ( $r = -.18$ ) were associated with lower resilience.

Two further results matter for programme design. Information use was associated with more self-blame ( $r = .33$ ), whereas emotional support was associated with less ( $r = -.30$ ) (Nandra & Khurana, 2025). All associations were small to moderate and cannot show cause and effect.

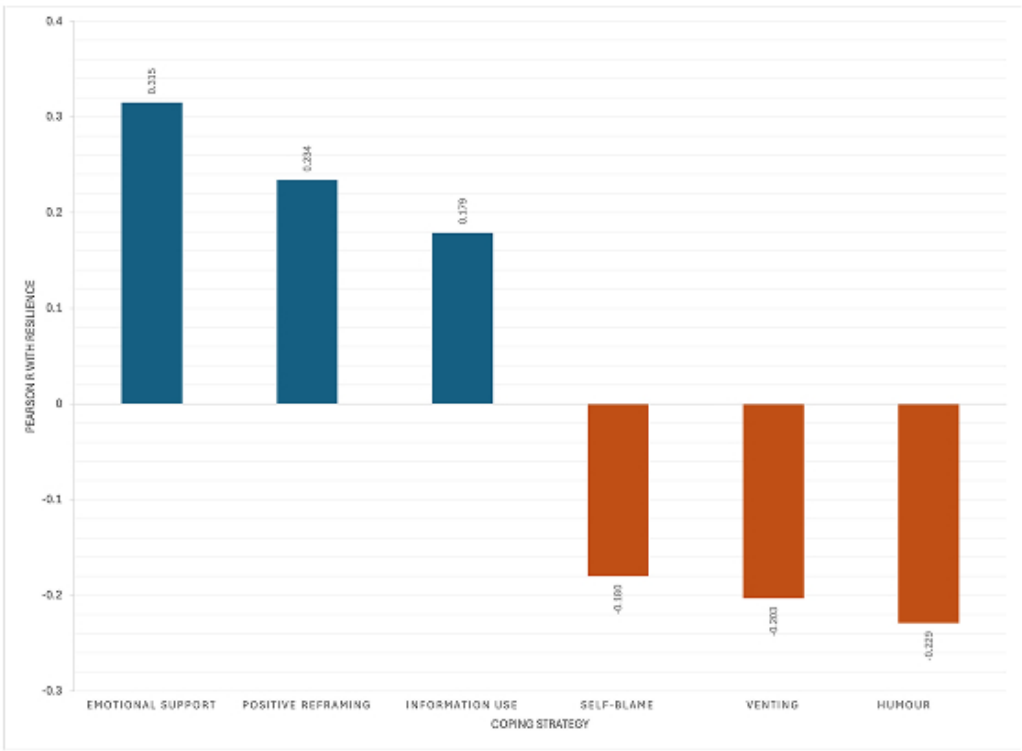


Figure 1. Coping strategies significantly related to resilience (Pearson r, N = 121). Data from Nandra and Khurana (2025).

## WHAT THE NATIONAL PROGRAMME ALREADY PROVIDES

The 2021 ART guidelines require symptom screening for depression at ART initiation and at every visit, with referral to the National Mental Health Programme in district hospitals or to psychiatry departments in medical colleges (NACO, 2021, pp. 111–112). The 2023 counsellor handbook lists internalised stigma alongside depression and anxiety as issues that can hamper adherence, and calls for regular screening and referral (NACO, 2023, Chapter 14). CSCs, which are linked to ART centres, run support group meetings, and outreach workers visit ART centres in rotation to give peer counselling to newly initiated patients (NACO, 2025, pp. 30, 65).

In the guidelines reviewed, the suggested support group topics centre on treatment, health and welfare information. None names coping skills or self-blame (NACO, 2025, p. 31). This brief does not assess how far these services are delivered in practice.

## RECOMMENDATIONS: PILOT AND EVALUATION

NACO's Care, Support and Treatment Division, working with State AIDS Control Societies, should approve a six-month controlled pilot of coping-focused support at three or four ART centres with linked CSCs, using existing counsellors and outreach workers, and decide on wider rollout from pre-specified criteria. (Duration and number of sites are indicative.

**Evidence base.** Stigma and depressive symptoms are associated with poorer ART adherence, and stigma appears to act partly by undermining adaptive coping and social support (Katz et al., 2013; Uthman et al., 2014). Psychosocial and peer-based interventions improve adherence on average, with small to moderate effects: Hedges'  $g = 0.37$  across 43 randomised trials of psychosocial interventions (Spaan et al., 2020), a standardised mean difference of 0.30 for adherence and 0.15 for viral load for psychological adherence interventions (Locher et al., 2019), and a network meta-analysis found that supportive and behavioural strategies such as peer support and counselling improved adherence compared with standard care (Kanters et al., 2017). NACO's own guidelines identify peer counselling for people struggling to cope with issues such as stigma and depression as an important retention intervention (NACO, 2021, p. 55). In our sample, self-blame ( $r = -.18$ ) and venting ( $r = -.20$ ) were associated with lower resilience, and positive reframing ( $r = .23$ ) and emotional support ( $r = .32$ ) with higher resilience. These are candidate content areas, not established mechanisms.

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**Option:** Continue as now

**Assessment:** No additional cost, but no evidence is generated on whether coping-focused content adds value. Suggested support group topics do not name coping skills or self-blame (NACO, 2025, p. 31).

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**Option:** Controlled pilot (recommended)

**Assessment:** Low cost, uses existing staff and meetings, and is proportionate to the evidence (one cross-sectional study,  $N = 121$ ).

3

**Option:** Immediate national rollout, including self-blame screening

**Assessment:** Not supported. Effects of comparable interventions are small, our evidence is correlational, and rollout would add workload before feasibility is known.

### Pilot design (indicative)

**Design:** prospective controlled pilot at three or four ART centres with linked CSCs, with matched comparison sites, over six months.

**Intervention:** a short structured module on managing self-blame and reframing difficult situations, delivered in CSC support groups and ART counselling by trained ART counsellors and CSC outreach workers. Some treatment education sessions would add structured peer-sharing time, to test whether pairing information with emotional support reduces self-blame. The link behind this (information use and self-blame,  $r = .33$ ) is exploratory and may reflect distressed people seeking more information.

**Outcomes (pre-specified):** primary outcomes are feasibility and acceptability (share of eligible patients reached, sessions delivered as planned, attendance). Secondary outcomes are resilience (Brief Resilience Scale), ART retention at six months, and viral load suppression where available. All outcomes are reported separately for women and men.

**Data:** routine programme records, including how many newly initiated patients received peer counselling and attended a support group (recorded at the existing monthly ART centre–CSC coordination meeting), plus a brief baseline and six-month survey. Ethical review and informed consent would be needed for any data beyond routine records.

**Sample size:** set by a formal power calculation that allows for clustering by site. Because pooled effects of similar interventions are small, a pilot of this size cannot confirm effectiveness. Its purpose is to test feasibility and to estimate effect sizes and between-site variation for a definitive cluster-randomised trial.

Decision rule. Before the pilot starts, NACO and the State AIDS Control Societies set progression criteria for feasibility, acceptability, and the direction and size of effects on resilience and retention. If they are met, proceed to a larger cluster-randomised evaluation across more states. If not, stop or redesign.

## LIMITATIONS

This brief draws on one cross-sectional, correlational study, so it cannot show that any coping strategy causes resilience, and the associations were small to moderate. The sample was small and recruited by convenience from three hospitals. Only 8 of 121 participants were women, and participants needed a basic understanding of English, so the findings may not generalise beyond male, English-comprehending patients in public hospitals in north India. The review of national guidelines is based on documents alone and does not show how services operate in practice. The recommendations are therefore for piloting and evaluation, not immediate scale-up. No correction was made for multiple comparisons, so smaller associations should be read cautiously.



## References